

Introduction to the *JACR* Appropriateness Criteria November 2021 Supplement

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The *JACR* November 2021 supplement contains 18 evidence-based topics from the ACR Appropriateness Criteria® (AC) to provide guidance to physicians regarding the best medical imaging study that should be ordered for a specific patient workup. For this update, 13 of the topics are new.

Updates in this edition provide guidance in two topics addressing back pain and another for plexopathy. The widespread prevalence of low back pain, a CMS-designated Priority Clinical Area, in the population makes this an important topic that frequently needs guidance for appropriate imaging. The updates to the topic include changes to the clinical scenarios and the addition of procedures that

strengthen the overall recommendation. In many of these patients, imaging is not necessary, but the AC address when imaging can help. The evaluation of adrenal mass update has new guidance for workup of these lesions, which can represent benign lesions or malignancy. The update for suspected aortic syndrome deals with the workup of a life-threatening condition.

Although a discussion of the 13 new topics would not be possible in this introduction, a few deserve mention. The topic addressing chronic cough is especially relevant in the current environment. With concerns surrounding the coronavirus 2019 pandemic, guidance for the evaluation of respiratory symptoms is especially important. In a patient with chronic cough for more than 8 weeks, the imaging recommendations differ dependent upon whether there has been unsuccessful empiric treatment. Epigastric pain is such a common patient presentation, and this topic was greatly needed.

The evaluation of breast screening in transgender patients is a first step to broader inclusion in the guidelines. Screening guidelines have been generally silent on the differences in screening requirements for this patient population. We hope to provide further expansion of guidelines for this population when the recommendations differ from the previously published guidelines for female patients. The recommendations vary based upon age, and the recommendations differ between transfeminine and transmasculine persons.

The overall AC address over 1,000 imaging modality variants and nearly 1,900 clinical scenarios, and we hope that these continue to provide relevant guidance for physicians as they consider which imaging is best for their patients.

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The ACR Appropriateness Criteria documents are updated regularly. Please go to the ACR website at www.acr.org/ac to confirm that you are accessing the most current content.

Disclaimer: The ACR Committee on Appropriateness Criteria and its expert panels have developed criteria for determining appropriate imaging examinations for diagnosis and treatment of specified medical condition(s). These criteria are intended to guide radiologists, radiation oncologists and referring physicians in making decisions regarding radiologic imaging and treatment. Generally, the complexity and severity of a patient's clinical condition should dictate the selection of appropriate imaging procedures or treatments. Only those examinations generally used for evaluation of the patient's condition are ranked. Other imaging studies necessary to evaluate other co-existent diseases or other medical consequences of this condition are not considered in this document. The availability of equipment or personnel may influence the selection of appropriate imaging procedures or treatments. Imaging techniques classified as investigational by the FDA have not been considered in developing these criteria; however, study of new equipment and applications should be encouraged. The ultimate decision regarding the appropriateness of any specific radiologic examination or treatment must be made by the referring physician and radiologist in light of all the circumstances presented in an individual examination.